Aisha

23 May 2026 - 20 August 2026 (90 days)

213 of 273 check-ins completed (78%) this period

Methodology on page 5

MIND DOMAIN SUMMARY

DOMAIN	BASELINE	CURRENT	TREND	ACTIVE PATTERN
Energy	5.0	**5.2**	->	sustained above baseline
Anxiety	5.5	**4.2**	v	sustained above baseline
Concentration	5.0	**5.2**	->	sustained above baseline
Mood	6.0	**7.3**	^	Within baseline range
Sensory overwhelm	4.0	**2.8**	v	Within baseline range

BODY DOMAIN SUMMARY - 70 DAYS LOGGED

DOMAIN	AVERAGE	RANGE	ACTIVE PATTERN
Fatigue across the day	4.1	**4-6**	No pattern detected
Joint & muscle pain	4.7	**4-5**	No pattern detected
Widespread pain	3.5	**3-4**	No pattern detected
Joint instability	4.9	**4-5**	Medication effect
Breathing	2.0	**2**	No pattern detected
Standing up	4.2	**4-5**	Time-of-day pattern
Gut	2.3	**1-4**	No pattern detected
Where you've landed	5.0	**5**	No pattern detected

PATTERNS DETECTED THIS PERIOD

Firm - enough data to stand on · Partial - worth watching · Early signal - limited coverage

Anxiety, Concentration, Energy - sustained above baseline FIRM
(15 days · from 4 Jul · firm data coverage)

Anxiety decreased by 1.4 points in the 21 days after Increased sertraline to 150mg. FIRM
(19 days before · 21 days after)

Anxiety & Concentration - move inversely PARTIAL
(58 overlapping check-ins · above correlation threshold)

4 additional lower-confidence patterns not shown here.

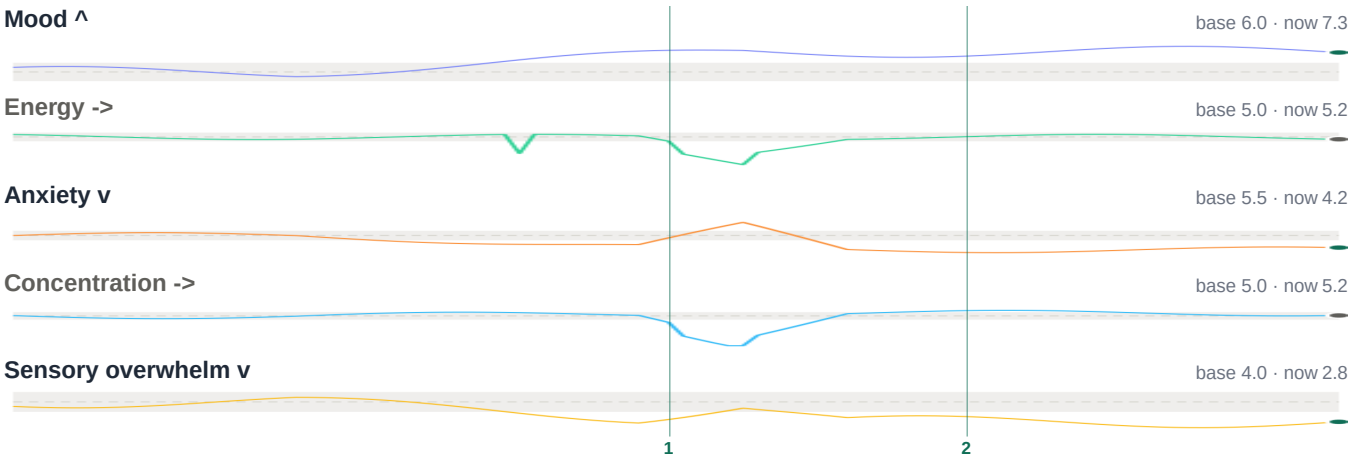
PATIENT WANTS TO DISCUSS

1. Could the anxiety and stomach flare from mid-July be linked to starting the ADHD assessment process?
2. Should I bring up the morning dizziness pattern before the cardiology referral?

SLEEP

Average sleep score	3.5 / 5 (personal baseline: 3.5)
Median hours slept	7.1 hours
Domain relationships	Fatigue across the day: worse after poor-sleep nights · Concentration: worse after poor-sleep nights

DOMAIN SPARKLINES



EVENT AND INTERVENTION TIMELINE

2 26 Jul · Therapy · Started hypermobility physiotherapy

Joint instability decreased by 0.6 points in the 14 days after Started hypermobility physiotherapy. (12 days before · 14 days after)

1 6 Jul · Life event · ADHD assessment referral letter arrived

4 Jul-18 Jul · Sustained deviation · Anxiety, Concentration, Energy

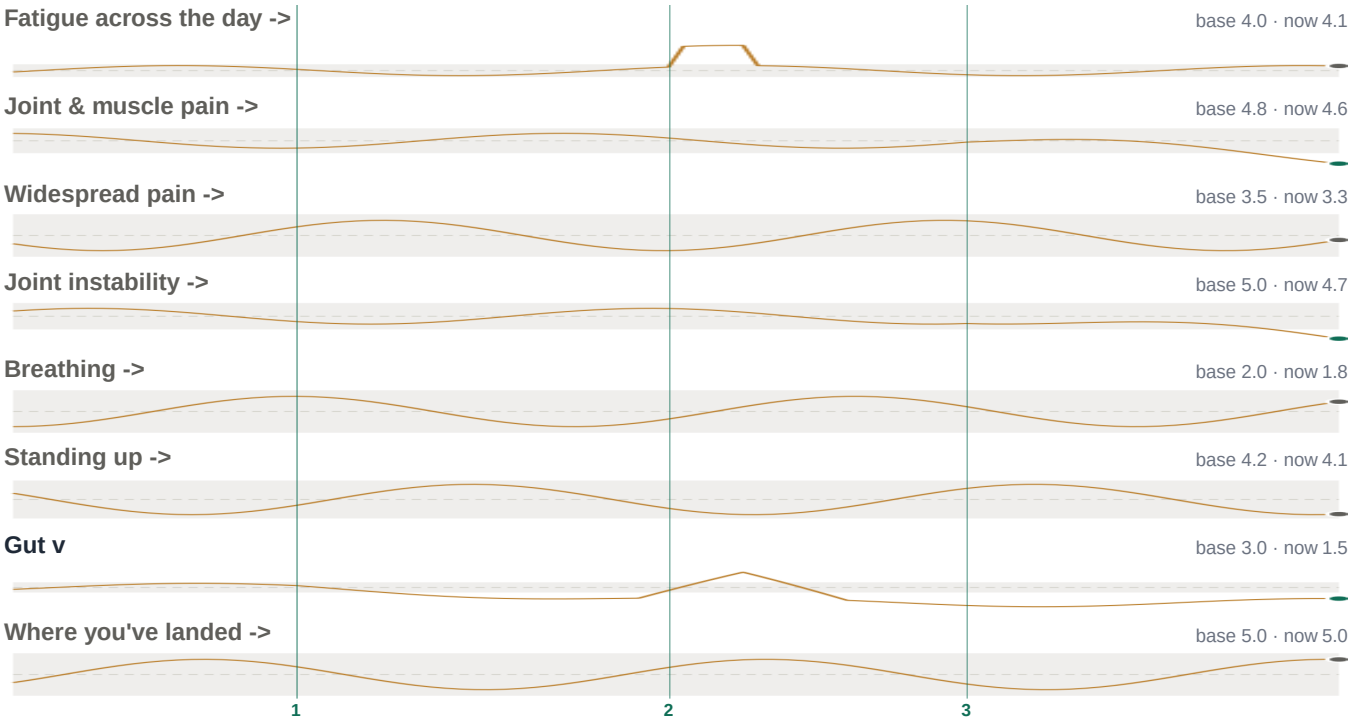
26 Jun · High variation · Energy

1 earlier entry not shown.

READING THIS PAGE

Grey = within personal baseline, not "fine" in an absolute sense: a score typical for this individual can still look high or low on a standard scale.

DOMAIN SPARKLINES



NOTABLE BODY EVENTS

DATE	EVENT	CONTEXT
31 May	A joint went out	Knee gave way going down stairs
26 Jun	Nearly fainted, or did	Stood up too fast after a shower, went grey and had to sit back down
30 Jun	Migraine started	-
14 Jul	A crash began	Pushed through the flare week and paid for it the next two days

2 earlier events not shown.

PAIN & INSTABILITY SITES

R Knee	**6 days**
L Shoulder	**4 days**
R Wrist	**3 days**

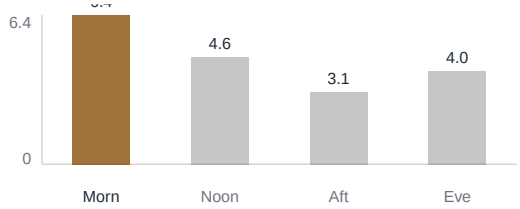
EVENT AND INTERVENTION TIMELINE

3. 26 Jul · Therapy · Started hypermobility physiotherapy
   Joint instability decreased by 0.6 points in the 14 days after Started hypermobility physiotherapy. (12 days before · 14 days after)
2. 6 Jul · Life event · ADHD assessment referral letter arrived
1. 11 Jun · Medication · Increased sertraline to 150mg
   Anxiety decreased by 1.4 points in the 21 days after Increased sertraline to 150mg. (19 days before · 21 days after)

> **READING THIS PAGE**
> Domain averages and active patterns are summarised on page 1.

TIME-BASED PATTERNS

ORTHOSTATIC - TIME OF DAY



No second pattern detected.

Highest: morning (avg 6.4), lowest: afternoon · range 3.3 pts

CONNECTIONS BETWEEN DOMAINS

DOMAIN PAIR	RELATIONSHIP	MIND CHECK-INS
Anxiety & Concentration	Move inversely - one high when other low	58

Threshold: min 14 overlapping check-ins · correlation not causation

MIND CHECK-IN COMPLETION - 213 OF 273 (78%)

WEEK	SCHEDULED	COMPLETED	%	
23 May - 29 May	21	17	**81%**	
30 May - 5 Jun	21	17	**81%**	
6 Jun - 12 Jun	21	17	**81%**	
13 Jun - 19 Jun	21	17	**81%**	
20 Jun - 26 Jun	21	17	**81%**	
27 Jun - 3 Jul	21	17	**81%**	
4 Jul - 10 Jul	21	17	**81%**	
11 Jul - 17 Jul	21	17	**81%**	
18 Jul - 24 Jul	21	17	**81%**	
25 Jul - 31 Jul	21	17	**81%**	
1 Aug - 7 Aug	21	9	**43%**	*v low coverage*
8 Aug - 14 Aug	21	17	**81%**	
15 Aug - 21 Aug	21	17	**81%**	

Week of 1 Aug - 7 Aug had unusually low completion (43%). Findings spanning this week should be interpreted with lower confidence.

METHODOLOGY

All data in this report is self-reported by the patient via brief, in-the-moment check-ins (ecological momentary assessment) rather than retrospective recall: several times a day for mind domains, once daily for body and sleep. All pattern detection and computation runs entirely on the patient's device: no data is processed on a server, and no machine learning or population-level model is used anywhere in this pipeline.

Every deviation, baseline and pattern here is a comparison against that patient's own recent history, a personal baseline computed as the rolling median of their last 30 days of data, never against a population norm. Patterns are only surfaced once they clear a minimum data-coverage threshold; an under-logged pattern is withheld entirely rather than shown with lower confidence, so an absence of findings should not be read as an absence of symptoms. Domain correlations are reported only as a qualitative direction and confidence tier; the underlying correlation coefficient and significance value are deliberately withheld, so a threshold-gated pattern in one person's self-reported history isn't read with more authority than it warrants.

This report is a structured symptom diary, not a diagnostic tool or a clinical measurement, and correlation shown here never implies causation. Findings are limited to the stated reporting window (2026-05-23 to 2026-08-20). For the full methodology, including exact statistical thresholds, data-quality gating rules and the evidence base behind these design decisions, visit symetric.app/for-clinicians.